DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 61–70.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 61: The court interpreter

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Mireille, 38, a court interpreter and mother of a 10-year-old son, on an inpatient psychiatric unit two days after admission. Her older brother, Wesner, and the unit social worker, who has her past hospital records, speak too.

> *"Thank you for coming, but I'm not sick. I've been given a gift, and the doctors here are slowing me down."*

If asked about…

When did this start?	About nine days ago. I woke at three in the morning knowing what I was meant to do, and I haven't needed sleep since.
What does the voice say?	It's an angel, a woman's voice, as clear as yours. She says my hands can heal people. I hear her all day now.
Sleep	Two hours, maybe. I'm not tired at all. My brother says I was up all night writing letters to the hospital board.
Mood and energy	I feel wonderful. My thoughts move so fast I can't write them down quickly enough.
Past hospital stays	At 32 and at 35. Both times the same: no sleep, big plans, the angel. Each time, once I was sleeping again, she went quiet within a few days.
Between episodes	I'm normal. I interpret in court full time and pack my son's lunches. I don't hear anything, not ever, not even for a day.
Low spells	Once, at 29, after my mother died. A month in bed. No voices then.
Alcohol, drugs, medications	No alcohol, no drugs. I stopped the monthly shot after a year because I felt fine. I take nothing now.
Medical history	I'm healthy. They checked my thyroid and my urine here and said it's normal. No head injuries, no seizures.
Thoughts of suicide or harming others	I would never hurt myself or anyone. The angel only tells me to heal. I gave the furniture away because I'll be too busy healing people to need it. I don't want to die; I have too much to do.
Family history	My aunt in Port-au-Prince had spells where she didn't sleep and preached in the market for days. Then she'd be fine for years.
Culture and faith	I'm Catholic and I pray every day, but praying is me talking to God. This voice talks out loud to me, and only when I'm not sleeping. My brother knows the difference too.

Only if asked (don’t volunteer)

- Each discharge summary says the voice faded within a few days as sleep and mood returned to normal
- She has never heard the voice while her mood was level
- She stopped the monthly antipsychotic injection after a year
- One depressive episode at 29, without psychosis
- An aunt had similar episodes with long well periods

Patient 61: answer and debrief

DIAGNOSIS

Bipolar I disorder

Mania with a voice and grandiose beliefs, plus a past schizoaffective label, makes schizoaffective disorder a real contender. Clue 5 flips it: in every episode the voice came and went with the mania, and she has never been psychotic while her mood was level, so she has never had the 2 or more weeks of psychosis without a mood episode that schizoaffective disorder requires. This is bipolar I disorder, current episode manic, with mood-congruent psychotic features. Exam lesson: a past label is a hypothesis, not a fact; rebuild the timeline of psychosis against mood from records and collateral.

What had to be ruled out

- Schizoaffective disorder: psychosis has never lasted 2 weeks or more without a mood episode; the voice clears within days as mood settles
- Schizophrenia: psychosis occurs only during mood episodes, and she functions fully between them
- Substance/medication-induced bipolar disorder: no alcohol, drugs or medications; urine drug screen negative
- Bipolar and related disorder due to another medical condition: normal thyroid tests and no medical illness
- Bipolar II disorder: psychosis and hospitalization make these episodes manic, not hypomanic

Checkpoint: What would you ask or check NEXT?

A. Accept the earlier schizoaffective diagnosis, since a hospital made it after a full workup

B. Get her past discharge summaries and ask her brother whether the voice ever continued after her sleep and mood returned to normal **(best)**

C. Recommend restarting the monthly antipsychotic shot before any further assessment

Bipolar I with psychotic features and schizoaffective disorder are separated by one fact: whether psychosis ever lasts 2 weeks or more while mood is level. Records and collateral answer that. A treats an old label as settled when the timeline was never checked. C is a treatment decision made before the diagnosis is clear, and it skips the assessment.

The case in six clues (as the app reveals it)

1. I feel better than I have in years, and I have important work to do. Every hour on this unit is an hour wasted.
2. Wesner: She has slept maybe two hours a night for nine days. She quit her job by email, gave her furniture to strangers, and says an angel's voice tells her that her hands can heal the sick.
3. I don't drink or use anything, and I take no medicines. Social worker: Her urine drug screen and thyroid tests here are normal. This is her third admission; at 32 and at 35 she was also hospitalized hearing a voice.
4. Wesner: The last hospital wrote schizoaffective disorder on her papers and sent her home on a monthly antipsychotic shot. She stopped it after a year because she felt fine.
5. Social worker: All three discharge summaries say the voice faded within a few days of her sleep and mood returning to normal. Wesner: Between spells she works full time and raises her son, and she has never heard that voice, not for a single day, when she is her usual self.
6. My only low spell was at 29, after our mother died. I stayed in bed for a month, but I never heard anything then. Apart from these spells I have been well for years at a time.

What a skilled clinician weighs after each clue

1. Bipolar I disorder, Substance/medication-induced bipolar disorder, Schizoaffective disorder
2. Bipolar I disorder, Schizoaffective disorder, Substance/medication-induced bipolar disorder
3. Schizoaffective disorder, Bipolar I disorder, Schizophrenia
4. Schizoaffective disorder, Bipolar I disorder
5. Bipolar I disorder, Schizoaffective disorder
6. Bipolar I disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder

Patient 62: The retired transit mechanic

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Desmond, 46, a former transit mechanic who lives in his own apartment, seen at home by an assertive community treatment (ACT) team social worker. His younger sister, Patrice, and the team nurse are there too.

> *"Patrice made me open the door. I don't have much to say. Everything I touch goes bad."*

If asked about…

When did this start?	About five years ago, at 41. I'd never had anything like it before. The first low spell took my job.
What does the voice say?	It's a man's voice, low and steady. When I'm down it says I'm worthless and I wrecked my family. Last spring it said the neighbors were reporting on me.
What happens to the voice when your mood is good?	It doesn't leave. Last spring I felt fine for almost two months, cooking, laughing, back at church, and it still talked every day.
Highs	Never. Not once. I've never gone days without sleep feeling great, and I've never felt like I could do anything.
Sleep and appetite	In the lows I sleep twelve hours and still feel tired, and I lose weight because I don't bother eating. In the steady stretches I sleep normal.
Alcohol, drugs, medications	I quit drinking twelve years ago, after my divorce. No drugs. I take the pills the team brings, one for mood and one for the voice.
Medical history	High blood pressure, that's it. They did an MRI of my head when this started, and it was normal. My blood work is fine.
Thoughts of suicide or harming others	Some days I think everyone would be better off without me. I don't have a plan and I won't do anything; my sister's kids need me. The voice has never told me to hurt myself or anyone else.
Family history	Our father had long spells in bed when we were kids. Nobody ever called it anything.
Work and daily life	I fixed buses for nineteen years. I've been on disability for four years now. The team helps me with groceries and bills when I'm low.
Culture and faith	I'm a Baptist. When I'm steady I usher on Sundays. My pastor visits when I'm low, and he's the one who told Patrice to call the team.

Only if asked (don’t volunteer)

- The voice continued daily for about seven weeks while his mood was steady
- Low spells have filled nearly all of the five years
- Passive thoughts that others would be better off without him; no plan or intent
- Father had long depressive spells

Patient 62: answer and debrief

DIAGNOSIS

Schizoaffective disorder

A voice and guilty beliefs during long, deep lows first suggest major depressive disorder with psychotic features. Clue 5 flips it: the voice kept going for about seven weeks while his mood was steady, more than the 2 weeks of psychosis without a mood episode that schizoaffective disorder requires. Clue 6 shows depressive episodes fill most of the illness, which separates schizoaffective disorder, depressive type, from schizophrenia. Exam lesson: in MDD with psychotic features, psychosis appears only inside the episode, so ask what happens to the voices when mood lifts, and ask about suicide whenever depression and voices occur together.

What had to be ruled out

- Major depressive disorder with psychotic features: the voice persisted about seven weeks with a steady mood
- Schizophrenia: major depressive episodes fill most of the illness
- Bipolar I disorder: no manic episode ever
- Substance/medication-induced psychotic disorder: no alcohol for twelve years, no drugs
- Psychotic disorder due to another medical condition: normal labs, thyroid and brain MRI
- Persistent depressive disorder: full major depressive episodes with psychosis, and the voice continues outside them

Checkpoint: What would you ask or check NEXT?

A. Ask him directly about thoughts of suicide and whether the voice ever tells him to harm himself **(best)**

B. Tell Patrice this sounds like depression with psychotic features and give her some reading about it

C. Ask the team psychiatrist to change his medication, since the low spells keep coming back

Deep depression with a voice calling him worthless raises suicide risk, so safety comes before diagnosis or treatment. B names a diagnosis before the timeline of the voice and his mood is known. C is a treatment change made before any assessment, and the prescribing decision is not the social worker's to make.

The case in six clues (as the app reveals it)

1. I've been down a long time. Most days I don't see the point of getting dressed.
2. Patrice: When he gets low like this, he hears a voice telling him he's worthless and that he ruined the family. He believes he did, and he stays in bed for weeks.
3. I've never had a time of feeling high or racing, and I quit drinking twelve years ago. Nurse: His labs, thyroid included, are normal, and his brain MRI was normal when all this began about five years ago.
4. Nurse: In those five years he has had three long low spells, including this one. The steady stretches between them lasted about two months each.
5. Nurse: During the last steady stretch his mood was fine for about seven weeks. He cooked, joked, slept well and went back to church, but the voice kept talking every day, now saying the neighbors were reporting on him. He was sure they were, and he kept the blinds shut all day.
6. Patrice: I keep his appointment calendar. In five years he has been in a low spell for nearly all of it, and the voice has been there in the lows and in the breaks alike.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Persistent depressive disorder, Bipolar II disorder
2. Major depressive disorder, Schizoaffective disorder, Schizophrenia
3. Major depressive disorder, Schizoaffective disorder
4. Major depressive disorder, Schizoaffective disorder
5. Schizoaffective disorder, Schizophrenia
6. Schizoaffective disorder

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder

Patient 63: The retired principal

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Loretta, 71, a retired elementary school principal, at a behavioral health visit in her primary care clinic after her third emergency room visit this month. Her husband, Calvin, and the clinic nurse speak too.

> *"I told Calvin there's no point in this. The doctors can't find it, but I know it's there."*

If asked about…

When did this start?	The disease? A few weeks ago I understood what it was. Calvin says I stopped being myself before that, maybe two months ago.
What do you believe is happening?	There's a sickness in me that no test can find, and it's passing to my grandchildren when I touch them. I'm being punished.
Mood	I'm not sad. I'm empty. Nothing tastes like anything and nothing matters.
Sleep	I wake at four every morning and lie there. I'm tired all day.
Appetite and weight	I don't want food. Calvin says I've lost fifteen pounds. My clothes hang on me.
Interests	I quit my quilting circle. I used to cook Sunday dinner for twelve; now Calvin does it and I just sit.
Alcohol, drugs, medications	I don't drink. I take one blood pressure pill, the same one for nine years. Nothing new.
Medical history	Blood pressure, a knee replacement four years ago. They've done every test this month and say I'm healthy. They're wrong.
Thoughts of death or suicide	Sometimes I think the babies would be safer if I were gone. I haven't planned anything, and I wouldn't do that to Calvin. We don't keep a gun in the house.
Family history	My brother had what we called a nervous breakdown in his fifties. He got treatment and came back to himself.
Past episodes	Never. I've never been like this, and I've never had ideas like this or heard things.
Culture and faith	We belong to a Methodist church. Our pastor prayed with me and then told Calvin to bring me here. Our church doesn't teach that God punishes grandchildren for their grandmother. That's my own understanding.

Only if asked (don’t volunteer)

- Low interest, early waking and weight loss began about two months ago, weeks before the disease belief
- Passive thoughts that the grandchildren would be safer if she were gone; no plan; no firearm at home
- No prior psychiatric episodes
- Her faith community does not share the belief

Patient 63: answer and debrief

DIAGNOSIS

Major depressive disorder

Repeated ER visits for a hidden disease first raise illness anxiety disorder, and her unshakable certainty moves the lead to delusional disorder, somatic type. Clue 5 flips it: her husband describes a full major depressive episode (lost interest, early waking, slowing, a 15-pound weight loss) that began about three weeks before the belief, and the belief is about guilt and deserved punishment. 'I'm not sad' does not rule out depression, because loss of interest or pleasure can stand in for depressed mood. Delusional disorder requires any mood episodes to be brief compared with the delusion; here the delusion exists only inside the episode, so this is major depressive disorder with mood-congruent psychotic features. Exam lesson: certainty separates a delusion from illness anxiety, and timing against mood separates psychotic depression from delusional disorder; when an older adult denies sadness, ask about interest, sleep and weight, ask directly about suicide, and arrange urgent psychiatric care.

What had to be ruled out

- Delusional disorder: the delusion arose within a major depressive episode and is confined to it
- Schizoaffective disorder: no psychosis ever outside a mood episode
- Illness anxiety disorder: the belief is held with delusional certainty, sits inside a depressive episode, and has lasted about five weeks, short of the 6 months illness anxiety requires
- Somatic symptom disorder: no distressing physical symptoms drive the worry
- Major neurocognitive disorder: normal memory screen; oriented and attentive
- Psychotic or depressive disorder due to another medical condition: normal blood work and scans
- Substance/medication-induced depressive disorder: no alcohol; one long-standing blood pressure medication
- Bipolar I disorder: no high or keyed-up spell ever

Checkpoint: What would you ask or check NEXT?

A. Show her the normal test results again and explain that the grandchildren are not at risk

B. Refer her to an infectious disease specialist for more testing to settle the worry

C. Ask directly about her mood, sleep, appetite and interest, and about thoughts of death or suicide **(best)**

Beliefs of guilt and punishment in an older adult can signal a severe depression, and depression with psychotic features carries high suicide risk, so assess mood symptoms and safety now. A argues with a fixed belief, which does not change it and can damage trust. B adds tests after a normal workup, which can reinforce the belief and delays the assessment she needs.

The case in six clues (as the app reveals it)

1. Calvin: Three times this month she's gone to the ER sure something terrible is wrong inside her body.
2. I have a disease the doctors can't find, and I've passed it to my grandbabies. I won't hold them anymore. Nothing anyone shows me changes that.
3. Nurse: Her blood work, scans and a memory screen are all normal, and she knows the date and follows every word. She takes the same blood pressure pill she's taken for years, and she doesn't drink.
4. I'm not sad. I'm being punished. I was a selfish mother, and God is letting this sickness spread from me to them so I'll see what I've done.
5. Calvin: She says she isn't sad, but about two months ago she quit her quilting circle, started waking at four, and began sitting for hours barely moving, too tired even to start dinner. She's lost fifteen pounds, can't enjoy even the grandkids' visits, and the disease talk only began about three weeks after all that.
6. Calvin: We've been married forty-four years. She has never had an idea like this before or heard things, never had a low or keyed-up spell, and she ran a school of six hundred children with a steady hand.

What a skilled clinician weighs after each clue

1. Illness anxiety disorder, Somatic symptom disorder, Delusional disorder, Major depressive disorder
2. Delusional disorder, Illness anxiety disorder, Major depressive disorder
3. Delusional disorder, Major depressive disorder
4. Delusional disorder, Major depressive disorder, Schizoaffective disorder
5. Major depressive disorder, Schizoaffective disorder, Delusional disorder
6. Major depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder

Patient 64: The hairstylist's referral

Setting: Brielle, 26, a hairstylist, at an intake for an outpatient DBT program. Her former therapist referred her. Her roommate and best friend, Tamar, came with her.

> *"I'm here because my old therapist said I need DBT. I'll do whatever. I just want to stop crashing."*

If asked about…

How long do the ups last?	Five or six days each time. Three times in the last two years. Then I'm either back to normal or I slide down.
What sets them off?	Nothing I can find. The last one started on a quiet Tuesday. It's not like someone hurt my feelings.
Sleep during the ups	Three hours a night and I feel amazing. I'm not lying awake upset; I just don't need it.
The current low	About a month. Bed most of the day, no appetite, I can't focus with a client in my chair. I don't even want to draw, and I used to sketch every night.
Thoughts of suicide or self-harm	I've never hurt myself and I don't want to die. I just want to feel like me again.
Relationships	When I crash I cancel on everyone for weeks, and when I'm up I talk over people and make promises I can't keep. That's what I meant by wrecking things. The breakup was mutual. I'm sad, but I'm not chasing her or begging. I've had the same close friends since middle school.
Alcohol, drugs, medications	A drink or two maybe twice a month. No drugs. I've never been on an antidepressant or ADHD meds. I take nothing.
Medical history	Healthy. My doctor checked my thyroid last month and it was normal.
Family history	My dad had years where he'd start three businesses in a month and then not get off the couch for weeks. He never saw anyone about it.
Work	When I'm low I cancel clients. When I'm up my manager loves me. I've never been fired or written up.
Culture and context	My family's close. My mom wasn't sure about therapy at first, but she's the one who told me to get a second opinion about the borderline label.

Only if asked (don’t volunteer)

- Highs come without any interpersonal trigger
- Sleep drops to three hours during highs, without tiredness
- Father had a similar pattern of highs and lows
- Never taken an antidepressant or a stimulant

Patient 64: answer and debrief

DIAGNOSIS

Bipolar II disorder

A prior borderline label, a breakup and impulsive spending make borderline personality disorder the first guess. Clue 4 flips it: her highs arrive without an interpersonal trigger, last five or six days, and bring less need for sleep without fatigue. That is a hypomanic episode: 4 days or more, a change others notice, and no marked impairment, hospitalization or psychosis. Hypomania plus a current major depressive episode and no lifetime mania is bipolar II. Exam lesson: borderline shifts last hours and follow interpersonal stress, while bipolar episodes last days and carry their own sleep change; ask about past highs before any antidepressant, and screen depressed clients for suicide at every visit.

What had to be ruled out

- Borderline personality disorder: no self-harm, no fear of abandonment, stable long friendships; the highs are days-long episodes, not hours-long reactions
- Bipolar I disorder: no episode with marked impairment, hospitalization or psychosis
- Cyclothymic disorder: full hypomanic and major depressive episodes have occurred
- Major depressive disorder: three past hypomanic episodes
- Substance/medication-induced bipolar disorder: no drugs, no antidepressants or stimulants, light drinking
- Bipolar and related disorder due to another medical condition: normal thyroid

Checkpoint: What would you ask or check NEXT?

A. Ask how long the up periods last, how she sleeps during them, and what, if anything, sets them off **(best)**

B. Start her in the DBT skills group, as the referral requested

C. Help her make a budget and coach Tamar on setting limits around the spending

Length, sleep and triggers are what separate bipolar highs from borderline mood shifts, so ask before accepting the referral label. B treats a diagnosis that has not been confirmed. C manages one behavior without assessing what is driving it.

The case in six clues (as the app reveals it)

1. My last therapist said I'm borderline. My moods swing hard, and I've wrecked things with people.
2. Right now I'm at the bottom. For about a month I've barely left bed, I can't focus on my clients, I skip meals, and I feel like garbage. My girlfriend and I split up right before it started.
3. Tamar: When she's up, she's a different person. She texted forty people about opening her own salon, booked four extra clients a day and bought a styling chair she didn't need.
4. The ups come out of nowhere, not after a fight or a breakup. They last five or six days, and I sleep three hours and wake up buzzing, not tired. Tamar: It's happened three times in two years, and each time her manager just thought she was on fire.
5. I've never been in a hospital or seen or heard things that weren't there. Between the ups and downs I'm steady for months. I don't hurt myself, I'm not scared of people leaving me, and Tamar and I have been best friends since sixth grade.
6. My doctor checked my thyroid last month and it was normal. I don't use drugs, I drink maybe twice a month, and I've never taken an antidepressant or a stimulant.

What a skilled clinician weighs after each clue

1. Borderline personality disorder, Bipolar II disorder, Cyclothymic disorder, Major depressive disorder
2. Borderline personality disorder, Major depressive disorder, Bipolar II disorder
3. Borderline personality disorder, Bipolar II disorder, Cyclothymic disorder
4. Bipolar II disorder, Borderline personality disorder, Bipolar I disorder
5. Bipolar II disorder, Borderline personality disorder
6. Bipolar II disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder; DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder

Patient 65: The mood chart

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Corinne, 45, a freelance graphic designer, at a behavioral health visit in her primary care clinic. Her wife, Maren, joins her.

> *"I'm on a mood stabilizer and I'm still a mess. Either the pill is wrong or I am."*

If asked about…

What did the urgent care doctor say?	He listened for ten minutes, said 'bipolar,' and gave me a prescription. Nobody has asked me much since.
How long does a high or low last?	Hours. I can be on top of the world at dinner and on the bathroom floor by midnight. By morning it's gone. I've never been up for days at a time.
Sleep	When I'm up all night it's because I'm replaying what someone said. I'm wrecked the next day. I've never felt rested on three hours.
What sets the moods off?	Someone not texting back, Maren being late, a client going quiet. It feels like proof I'm about to be dropped.
Self-harm and past attempts	I cut in high school. At 24 I swallowed a bottle of pills after a breakup and woke up in the ICU. I haven't cut in years.
Thoughts of suicide now	When we fight I sometimes think everyone would be better off without me. Right now I have no plan and I don't want to die. Maren keeps my pills in a lockbox; we agreed on that.
Alcohol, drugs, medications	I quit drinking at thirty because it made the fights worse. No drugs. Just the mood stabilizer from urgent care.
Medical history	Migraines. My doctor checked my thyroid and blood work last month, and both were normal.
Family history	My mother had a temper nobody could predict, and she left when I was twelve. My dad never talked about feelings at all.
Relationships and work	Friends come and go. I fall hard and then they let me down. I've lost nine clients in two years, some after I sent emails I regret.
Sense of self	I don't know who I am half the time. I've changed careers three times and redo my whole look every year.

Only if asked (don’t volunteer)

- The mood chart shows no shift lasting past the next morning
- No high has ever lasted even two days
- Sleepless nights bring exhaustion the next day
- Suicide attempt by overdose at 24 with an ICU stay
- Pills are kept in a lockbox by agreement

Patient 65: answer and debrief

DIAGNOSIS

Borderline personality disorder

Fast talk, spending sprees, sleepless nights and a prior bipolar label point to bipolar II at first. Clue 4 flips it: her mood chart shows no high or low lasting past the next morning, the shifts follow perceived rejection, and her sleepless nights bring exhaustion, which is insomnia, not the reduced need for sleep of hypomania. Self-harm since adolescence, a past attempt, emptiness, intense anger, idealizing and devaluing, and frantic efforts to avoid being left fit borderline personality disorder. Exam lesson: track how long each shift lasts, what set it off and how the client feels the next day; assess suicide risk every time, and offer DBT, the first-line treatment.

What had to be ruled out

- Bipolar II disorder: no 4-day hypomanic episode in her lifetime; every shift lifts by the next morning and follows a trigger
- Bipolar I disorder: no manic episode
- Cyclothymic disorder: the highs are hours-long reactions to rejection, not hypomanic periods, and sleep loss brings exhaustion
- Dependent personality disorder: she meets the threat of being left with rage and emptiness, not appeasement
- Substance/medication-induced or medical cause: no alcohol or drugs; normal thyroid and labs

Checkpoint: What would you ask or check NEXT?

A. Tell her the chart does not fit bipolar disorder and suggest she stop the mood stabilizer

B. Refer her to a DBT program and schedule a follow-up in a month

C. Ask about current thoughts of suicide or self-harm, any plans, and access to pills or other means **(best)**

A past near-lethal attempt is the strongest predictor of a future one, so current risk comes first. A goes beyond the social worker's role, and stopping a medication abruptly is the prescriber's decision. B is a good next step later, but it skips the risk assessment and leaves a month without contact.

The case in six clues (as the app reveals it)

1. An urgent care doctor told me last year that I'm bipolar. The pill he started hasn't touched it.
2. I get these highs where I talk fast, buy things online at 2 a.m. and stay up all night. Then I crash and want to disappear.
3. Maren: The highs usually start after something feels like a rejection, like a client not answering or me working late. She can go from furious to giddy to sobbing in one evening.
4. I brought the mood chart my doctor asked for: six weeks, rated three times a day. Maren: Not one high or low lasted past the next morning, and on the nights she's up, she's lying awake replaying a fight and is exhausted the next day. In eleven years together I've never seen a high last even two days.
5. This has been me since I was fifteen. I cut myself in high school, and at twenty-four I took pills after a breakup and ended up in intensive care. I still feel empty, like nobody's home inside me.
6. Maren: In the same week I'm the love of her life and the worst person alive. If I visit my sister overnight, she begs me not to go and texts until dawn. Her thyroid and labs are normal, and she doesn't drink or use drugs.

What a skilled clinician weighs after each clue

1. Bipolar II disorder, Bipolar I disorder, Cyclothymic disorder, Borderline personality disorder
2. Bipolar II disorder, Bipolar I disorder, Borderline personality disorder
3. Borderline personality disorder, Bipolar II disorder, Cyclothymic disorder
4. Borderline personality disorder, Bipolar II disorder
5. Borderline personality disorder, Bipolar II disorder
6. Borderline personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder; Stanley & Brown (2012), Cognitive and Behavioral Practice 19, Safety Planning Intervention

Patient 66: The farm equipment salesman PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Gideon, 36, a regional sales manager for a farm equipment dealer, at his gastroenterology clinic, where the nurse asked the behavioral health social worker to see him. His wife, Hanna, came with him.

> *"Honestly, I feel great. My wife's the one who's worried. I've got a lot of ideas I want to get moving on."*

If asked about…

When did this start?	About two weeks ago. A few days after I started the prednisone I just woke up full of plans.
Mood and energy	Never better. My mind's going a hundred miles an hour. Hanna says I interrupt everyone.
Sleep	For a while three hours, and I wasn't tired. This week it's more like five or six.
Spending and plans	I ordered an eighteen-foot fishing boat online and told my boss we could triple sales by spring. I canceled the boat. The plan was a bit much.
Medications	Prednisone for my colitis, a high dose at first. The doctor's been stepping it down every few days. That's the highest dose I've ever taken.
Alcohol and drugs	I don't drink, never have. No drugs. They tested my urine this week, and it was clean.
Medical history	Ulcerative colitis since I was 28. Usually it's quiet. They checked my thyroid this week and it was normal.
Thoughts of suicide or harming others	No, nothing like that. Hanna says I drove too fast to a customer meeting last week. I'd never hurt anyone.
Past mood episodes	Never. I've never been really down for weeks or wound up like this. I'm the calm one.
Family history	Nobody on either side that we know of. My dad's the steadiest man alive.
Work and functioning	My boss called Hanna after my pitch meeting and told me to take some days off. That's never happened before.
Culture and context	We live outside a small farm town where people talk, so Hanna wanted to do this here at the GI clinic and not at the mental health center.

Only if asked (don’t volunteer)

- High-dose prednisone began about two and a half weeks ago; symptoms started four days later
- Symptoms are easing as the dose is tapered
- His boss sent him home from work after the pitch meeting
- No previous mood episodes and no family history

Patient 66: answer and debrief

DIAGNOSIS

Substance/medication-induced bipolar disorder

Two weeks of elevated mood, little sleep, grandiose plans and spending look like a first manic episode of bipolar I. Clue 4 flips it: the change began four days after high-dose prednisone, a medication known to cause mania, and clue 5 shows it fading as the dose comes down, with no prior episodes. That is a substance/medication-induced bipolar disorder. Contrast the DSM rule: a full manic episode that begins during antidepressant treatment and persists beyond the drug's physiological effect counts as bipolar I. Safety: steroids are never stopped abruptly; the prescriber manages the taper, and if the mania persists after the drug is gone, reconsider an independent bipolar disorder.

What had to be ruled out

- Bipolar I disorder: onset followed high-dose prednisone within days and is fading with the taper; no prior episodes
- Stimulant intoxication: no drug use; urine drug screen negative
- Delirium: alert, oriented and following conversation
- Bipolar and related disorder due to another medical condition: normal thyroid; colitis is not a recognized cause of mania
- Bipolar II disorder: no major depressive episode ever, and being sent home from work is marked impairment, beyond hypomania

Checkpoint: What would you ask or check NEXT?

A. Advise him to stop the prednisone today, since it seems to be causing the change

B. With his consent, contact his GI doctor so the prescriber knows about the mood change and can review the steroid plan **(best)**

C. Record bipolar I disorder, since he meets the criteria for a manic episode

A medication that can cause mania is in play, so the prescriber needs to know and decide on the plan. A is dangerous: high-dose steroids must not be stopped suddenly, and medication decisions belong to the prescriber. C ignores the medication rule-out that DSM requires before diagnosing a primary bipolar disorder.

The case in six clues (as the app reveals it)

1. Hanna: For about two weeks he's been someone else. He's up at four reorganizing the garage, he talks so fast I can't keep up, and his boss just sent him home from work.
2. I feel incredible, better than I ever have. I pitched my boss a plan to triple our sales, and I ordered a fishing boat online. Three hours of sleep and I'm good to go.
3. I've never had anything like this, and I've never seen a psychiatrist. I don't drink or use drugs. Hanna: His urine drug screen and thyroid this week were normal, and he knows the date and follows every word.
4. Hanna: His GI doctor started him on a high dose of prednisone for a colitis flare about two and a half weeks ago. He started acting different about four days after the first dose.
5. The doctor has been lowering the dose every few days. Hanna: As it's come down, he's sleeping five or six hours, he canceled the boat, and he admits the sales plan was over the top. He's still chatty, but he's mostly himself.
6. Hanna: He's always been steady, through layoffs and our son's NICU stay. He's never had a spell of being low for weeks, and nobody in either family has had anything like this.

What a skilled clinician weighs after each clue

1. Bipolar I disorder, Substance/medication-induced bipolar disorder, Stimulant intoxication
2. Bipolar I disorder, Substance/medication-induced bipolar disorder
3. Bipolar I disorder, Substance/medication-induced bipolar disorder
4. Substance/medication-induced bipolar disorder, Bipolar I disorder
5. Substance/medication-induced bipolar disorder, Bipolar I disorder
6. Substance/medication-induced bipolar disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Substance/Medication-Induced Bipolar and Related Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder

Patient 67: The clock repairer

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Ezra, 37, who repairs watches and clocks in a workshop behind his mother's house, at a behavioral health visit in his primary care clinic. His mother, Mae, asked for the visit and joins him.

> *"My mother worries too much. I'm fine in my shop. It's people that are the problem."*

If asked about…

How long have you had these experiences?	Since I was a boy. At ten I told my teacher I could tell when the bell would break. Nothing's really changed since.
The clocks	When one stops by itself, somebody I know gets bad news. It's happened enough that I watch for it. I can't explain it.
Your grandfather	It's a feeling, like a draft, that he's in the corner. I don't see him or hear him. I know it probably isn't really him.
The song	It felt like it was aimed at me, about a customer I don't trust. Thinking about it now, probably it wasn't.
Mood	I'm not sad. Just tense around people, always have been.
Sleep	Fine. Seven hours. I work regular hours.
Alcohol, drugs, medications	I don't drink, never have. No drugs, no medicines.
Medical history	Healthy. Last year the doctor did blood work and a head scan because my mother asked. Both normal.
Thoughts of suicide or harming others	No. I don't want to hurt anyone or myself. I want to be left alone to work.
Work and daily life	I've fixed watches and clocks for fifteen years. Customers drop them through a slot in the door; I don't like them coming in. I cook and keep my own place.
Childhood development	Mae: He talked early, played pretend with his cousins and looked you in the eye. He was just odd, and other kids teased him.
Culture and family	Mae: We're Lutheran. Nobody in our family or our church believes in signs from clocks. He came up with all this on his own as a boy.

Only if asked (don’t volunteer)

- He doubts the presence and the song when asked
- The pattern has been stable since about age ten
- Normal early development
- His family and church do not share these beliefs

Patient 67: answer and debrief

DIAGNOSIS

Schizotypal personality disorder

A loner with odd beliefs and an uncle with schizophrenia first raises schizophrenia. Clue 4 flips it: his ideas of reference and sense of a presence are held loosely, fall short of delusions and hallucinations, and have been stable since childhood with work and self-care intact, so there is no psychotic episode and no decline. Magical thinking, unusual perceptual experiences, odd speech, eccentric dress and social anxiety that does not ease with familiarity fit schizotypal personality disorder; schizoid personality disorder has detachment without the odd beliefs and perceptions. Exam lesson: ask how firmly a belief is held and how long it has lasted, check it against the client's culture, and confirm a personality pattern with collateral, not from one session.

What had to be ruled out

- Schizophrenia: no delusions or hallucinations held with conviction, no decline, and a stable lifelong pattern
- Delusional disorder: the beliefs are not fixed; he doubts them when asked
- Schizoid personality disorder: he has odd beliefs, unusual perceptions and eccentric behavior
- Avoidant personality disorder: his social fear comes from suspicion, not from feeling inadequate, and he has odd beliefs
- Autism spectrum disorder: normal early language, pretend play and eye contact as a child
- Psychotic disorder due to another medical condition or substance: normal blood work and head scan; no alcohol or drugs

Checkpoint: What would you ask or check NEXT?

A. Refer him for an urgent antipsychotic evaluation, since his uncle has schizophrenia

B. Record schizoid personality disorder, since he has never had friends

C. Ask how sure he is about the song and his grandfather, what he actually sees or hears, and how long he has had experiences like these **(best)**

Whether these are delusions and hallucinations or looser odd beliefs and perceptions, and whether they are new or lifelong, decides between a psychotic disorder and a personality pattern. A acts on family history before the experiences themselves are assessed. B ignores the odd beliefs and perceptions, which schizoid personality disorder does not include.

The case in six clues (as the app reveals it)

1. Mae: He's 37 and has never had a real friend. He spends his days alone in the workshop.
2. I know when things are going to happen. When a clock in my shop stops on its own, someone I know is about to get bad news. Last week a song on the radio was meant for me, a warning about a customer.
3. Mae: His uncle has schizophrenia. For years Ezra has said he senses his late grandfather standing in the corner of the shop, and he talks in a roundabout way, circling a point without landing on it.
4. It's not a voice, and I don't see him. It's more a feeling that he's there, and the song probably wasn't really about me; it just felt that way. Mae: He's been like this since he was a boy, nothing has changed in years, and he's never stopped working or looking after himself.
5. People make me nervous, even the ones I've known for years. The mail carrier has come for ten years and I still go tense, because I don't trust what he's really up to.
6. Mae: He wears a wool coat in July and keeps rows of stones on the windowsill 'to keep the room balanced.' As a boy he talked early and played pretend with his cousins; he was just odd. His blood work and head scan last year were normal, and he doesn't drink or use anything.

What a skilled clinician weighs after each clue

1. Schizoid personality disorder, Avoidant personality disorder, Schizotypal personality disorder, Autism spectrum disorder
2. Schizophrenia, Schizotypal personality disorder, Delusional disorder
3. Schizophrenia, Schizotypal personality disorder
4. Schizotypal personality disorder, Schizophrenia
5. Schizotypal personality disorder, Avoidant personality disorder
6. Schizotypal personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Schizotypal Personality Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia

Patient 68: The third lock change

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Delphine, 64, a retired court reporter, meets the resident services social worker in her senior apartment building after asking the office, for the third time this year, to change her locks. Her niece, Lena, came with her.

> *"I'm not here for counseling. I'm here about my locks."*

If asked about…

What has been happening?	Things on my counter aren't where I left them. My mail came opened once. Someone has a key, I'm telling you.
How sure are you?	Sure? No. I can't prove it's the manager. Maybe it's nobody. But I've learned to expect the worst from people.
The manager	He smiles at me like there's a joke I'm not in on. People do that. They think I don't notice.
How long have you felt people can't be trusted?	My whole life. Even as a girl I knew better than to tell anyone anything.
Family	My sister cheated me over our mother's will. My brother made a crack about me at his daughter's wedding twenty-two years ago, and I haven't spoken to him since. Lena is the only one I let in.
Work	Thirty years as a court reporter. I never missed a deadline. Coworkers were always angling for my assignments behind my back.
Memory and health	My memory's fine. I do my own taxes. My doctor ran labs and a memory test this year, and he says I'm healthy. I didn't tell him about any of this.
Alcohol, drugs, medications	I don't drink. A cholesterol pill, that's all.
Sleep and mood	I sleep fine, with the chain on. I'm not depressed. I'm careful.
Thoughts of suicide or harming others	No. I'd never hurt anyone, and I don't want to die. I just want my door secure.
Culture and context	I was the oldest of five, and my father said trust no one outside the house. Then I spent thirty years in courtrooms listening to people lie under oath.

Only if asked (don’t volunteer)

- She is not sure anyone has entered and can't name who
- Lifelong distrust across family, work and services
- A twenty-two-year grudge against her brother
- She did not tell her doctor about her concerns

Patient 68: answer and debrief

DIAGNOSIS

Paranoid personality disorder

A specific claim that someone has a key first suggests a persecutory delusion. Clue 4 flips it: the suspicion has run for decades across family, work and services, with lasting grudges, and clue 5 shows she does not hold the belief with fixed certainty. Pervasive, long-standing distrust, hidden meanings read into a smile, reluctance to confide and grudges, without a fixed false belief, fit paranoid personality disorder. Exam lesson: delusional disorder has a fixed false belief held despite evidence, often newer; paranoid personality is a lifelong style of distrust. Check for real exploitation and for realistic, culturally grounded mistrust before labeling suspicion.

What had to be ruled out

- Delusional disorder: no single belief held with fixed certainty; she doubts it when asked
- No mental disorder (normal or expected reaction): the office logs show no entry, and the distrust spans decades and many people
- Major neurocognitive disorder: normal memory test; manages her own taxes
- Schizotypal personality disorder: no odd beliefs, unusual perceptions or eccentric behavior
- Schizophrenia: no hallucinations, disorganization or decline
- Substance or medical cause: normal labs, no alcohol

Checkpoint: What would you ask or check NEXT?

A. Ask how long she has felt this way about people, and whether she has had similar trouble trusting others in other places **(best)**

B. Refer her for an antipsychotic evaluation for a persecutory delusion

C. Show her the office key log until she accepts that no one has entered

With medical and cognitive causes checked, the open question is whether this is one new fixed belief or a long-standing style of distrust, and history answers that. B assumes a delusion before the pattern and her certainty are known. C argues with the belief, which rarely changes it and is likely to cost her trust.

The case in six clues (as the app reveals it)

1. I want my locks changed again. Someone in this building has a key to my place.
2. Things get moved. My mail came opened last month. And the manager smiles at me like he knows something I don't.
3. Lena: The office logs show no staff entry since the last lock change. Her memory is sharp, and she still does her own taxes. Her doctor did labs and a memory test this year, all normal, and she doesn't drink.
4. Lena: This isn't new, and it isn't just the building. The family says she's been this way since she was a girl. Over the years it was her sister cheating her over Grandma's will, a dentist overcharging her on purpose and coworkers plotting at every job, and she hasn't spoken to her brother in twenty-two years over a joke at a wedding.
5. I can't say for sure it's the manager. Maybe it's maintenance, maybe nobody, but you learn people are out for themselves. I don't tell anyone my business, not even my doctor.
6. Lena: She's never seen or heard things that weren't there, and she isn't odd or eccentric. She worked thirty years in the courts without missing a deadline, and her apartment is spotless.

What a skilled clinician weighs after each clue

1. Delusional disorder, Paranoid personality disorder, Major neurocognitive disorder, No mental disorder (normal or expected reaction)
2. Delusional disorder, Major neurocognitive disorder, Paranoid personality disorder
3. Delusional disorder, Paranoid personality disorder
4. Paranoid personality disorder, Delusional disorder
5. Paranoid personality disorder, Schizotypal personality disorder
6. Paranoid personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Paranoid Personality Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder

Patient 69: The ninth grader

Setting: Jasper, 15, a ninth grader, at his high school's health center. His English teacher sent a note, and his mother joins by phone from work.

> *"I don't know why I'm here. I'm just annoyed all the time. It's not a big deal."*

If asked about…

When did this start?	End of seventh grade, I guess. That's when everything started getting on my nerves.
Have there been good stretches?	Summer camp was okay for a couple of weeks. Then it was back to the same.
Sleep	I sleep fine. Like nine hours. I'm still tired.
Energy and focus	I'm tired all day. I read a page and have to read it again.
Things you enjoy	Basketball with my cousins on weekends. That's still fun. Video games sometimes.
Appetite	Normal. My mom says I eat like a horse.
How you see yourself	Kind of a loser. My brother's a senior and everybody loves him. I'm just the other one.
Thoughts of suicide or self-harm	No. I've never wanted to die or hurt myself. I'd tell my mom if I did, probably.
Alcohol, vaping, drugs	No. I don't vape or drink. They tested me for sports and it was clean.
Temper	I don't yell or throw stuff. I just say 'whatever' and go to my room.
Family history	Mother: My dad was gloomy his whole life. We didn't have a word for it back then.
Family and context	We moved here from Ohio in seventh grade after my parents split up. I see my dad on holidays. It's fine, I guess.

Only if asked (don’t volunteer)

- Low, irritable mood most days for about 18 months, with no good stretch longer than a few weeks
- Still enjoys basketball; sleep and appetite normal
- Moved after his parents separated in seventh grade
- Maternal grandfather had lifelong low mood

Patient 69: answer and debrief

DIAGNOSIS

Persistent depressive disorder

Irritability in a teen raises depression, DMDD and ODD. Clue 3 rules out DMDD: he has never had severe, frequent outbursts, and clue 4 dates the change to about age 13, after the before-10 onset DMDD requires. Clue 4 flips it from MDD: irritable mood most days for about 18 months, more than the 1 year required in youth, never lifting for more than a few weeks, with tiredness, poor focus and low self-esteem but too few symptoms for a major depressive episode. That is persistent depressive disorder, early onset. Exam lesson: in youth, irritable mood can stand in for depressed mood and the threshold is 1 year, not 2; keep screening for suicide and for a major depressive episode on top.

What had to be ruled out

- Major depressive disorder: too few symptoms for a major depressive episode; he still enjoys basketball, sleeps and eats normally
- Disruptive mood dysregulation disorder: no severe recurrent outbursts, and onset was after age 10
- Oppositional defiant disorder: occasional short replies, not a pattern of defiance, arguing or vindictiveness
- Adjustment disorder: about 18 months of symptoms that meet criteria for another disorder
- Cyclothymic disorder and bipolar disorders: no periods of being wired or needing less sleep
- Depressive disorder due to another medical condition or a substance: normal thyroid and blood count, clean drug screen

Checkpoint: What would you ask or check NEXT?

A. Refer him to an anger management group for snapping at classmates

B. Ask when the irritability and tiredness started, and whether he has had stretches of feeling like himself since then **(best)**

C. Set up a home behavior chart with consequences for disrespect

Duration and breaks decide between a depressive episode, a chronic depressive disorder and a passing reaction, so the timeline comes first. A treats irritability as a skills problem without asking what is behind it. C manages behavior and can miss depression, which in teens often shows up as irritability.

The case in six clues (as the app reveals it)

1. Teacher's note: Jasper has been short-tempered and checked out in class. He snapped at two classmates this month.
2. Everything annoys me. I'm tired all the time, and I can't focus when I read.
3. Mother: He's never thrown things or had screaming fits, even as a little kid. He gets short with us, says 'whatever' and goes to his room, but he still plays basketball with his cousins every weekend and eats like a horse.
4. Mother: This isn't new. He's been like this most days since the end of seventh grade, about a year and a half now, and the best stretch, at summer camp, lasted two or three weeks.
5. I sleep okay, and I don't want to die or anything. I just think I'm kind of a loser compared to my brother.
6. Mother: His pediatrician checked his thyroid and blood count three months ago, and both were normal. His sports physical drug screen was clean, and he's never had a stretch of being wired or needing less sleep.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Disruptive mood dysregulation disorder, Oppositional defiant disorder, ADHD
2. Major depressive disorder, Disruptive mood dysregulation disorder, Persistent depressive disorder
3. Major depressive disorder, Persistent depressive disorder
4. Persistent depressive disorder, Major depressive disorder
5. Persistent depressive disorder, Depressive disorder due to another medical condition
6. Persistent depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Persistent Depressive Disorder; DSM-5-TR (APA, 2022), Depressive Disorders: Disruptive Mood Dysregulation Disorder

Patient 70: The quiet resident

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Tiana, 31, a former call-center team lead, four months into a residential treatment program for women. The program nurse speaks, and her older brother, Darnell, joins by phone.

> *"I'm doing my program. I just don't want to sit in a room where everybody's thinking about me."*

If asked about…

When did the voices start?	Maybe three months before I came here. I was using every day then. I thought they'd stop when I quit. They didn't.
What do you hear?	Two women. They talk about me, what I'm wearing, what I'm doing wrong. Most days, mostly at night.
What do you believe is going on?	Back home my neighbors were filming me. Here, I think the vent is how they listen. I taped it.
Drug use	Meth, almost every day for about a year. Nothing since I got here four months ago. They test me twice a week.
Alcohol and medications	I don't drink much and never have. The doctor here started me on a pill last week. I don't think it does anything yet.
Medical history	Nothing big. The doctor did blood work and a scan of my head, and it was normal. No seizures, no head injuries.
Mood	Not sad. Not happy. I don't feel much of anything. I've never had a time of feeling on top of the world.
Sleep	I sleep a lot. I don't really want to get up.
Thoughts of suicide or harming others	No. I don't want to die, and the voices don't tell me to do anything. I'd never hurt the women here. I just stay away from them.
Work and daily life	I ran a team of twelve at a call center. Now Darnell says I don't call him unless he calls me. Staff remind me to shower. I quit job training.
Family history	Darnell says our grandmother 'heard things' and lived in a group home most of her life. Nobody talked about it.
Culture and context	Darnell's the only family I talk to. He drives three hours every visiting day. He says I used to be the funny one.

Only if asked (don’t volunteer)

- Psychosis began about three months before admission, during daily methamphetamine use
- Four months of twice-weekly negative urine tests in a controlled setting
- A grandmother with a chronic psychotic illness
- Started an antipsychotic one week ago

Patient 70: answer and debrief

DIAGNOSIS

Schizophrenia

Psychosis that began during heavy methamphetamine use first suggests a substance-induced psychotic disorder. Clue 4 flips it: voices and paranoia have persisted through four months of verified abstinence, well beyond the roughly 1 month after which DSM-5-TR says to consider an independent psychotic disorder. Hallucinations, persecutory beliefs and negative symptoms (flat face, little speech, poor self-care), a clear drop in functioning and seven months of illness, with normal tests and no mood episode, meet schizophrenia. Once the psychosis is judged independent of the drug, the 6-month clock counts the whole continuous illness (three months before admission plus four since), not just the clean months, so schizophreniform no longer fits. A stimulant use disorder, now in early remission in a controlled environment, is likely too; it is recorded alongside schizophrenia, not instead of it, because it cannot explain psychosis that outlasts abstinence. Exam lesson: substances can trigger or mask a primary psychosis, so recheck the diagnosis after a month or more of abstinence and treat both disorders together in integrated care.

What had to be ruled out

- Substance/medication-induced psychotic disorder: psychosis has persisted through four months of verified abstinence
- Psychotic disorder due to another medical condition: normal blood tests and brain scan
- Schizoaffective disorder and mood disorders with psychotic features: no manic or major depressive episode
- Schizophreniform disorder: total illness has lasted about seven months
- Social anxiety disorder and paranoid personality disorder: she hears voices and holds persecutory beliefs, with a clear decline from her usual self
- Stimulant use disorder: likely co-occurring (a year of near-daily use; in early remission, in a controlled environment, if full criteria were met); assess it fully and record it alongside, but it does not explain the persisting psychosis

Checkpoint: What would you ask or check NEXT?

A. Reassure her that the voices are leftover drug effects that will clear with more time sober

B. Make sure the program psychiatrist evaluates her for a primary psychotic disorder, with a medical workup, since the psychosis has outlasted abstinence by months **(best)**

C. Discharge her for missing groups, as the program rules require

Psychosis that persists well beyond about a month of verified abstinence points to an independent psychotic disorder and needs evaluation and treatment now. A delays care on a theory the timeline no longer supports. C punishes symptoms of an untreated illness, removes her from care and could leave her unsafe.

The case in six clues (as the app reveals it)

1. I don't go to group anymore. The other women look at me, and I know what they're saying in their heads.
2. Nurse: She came to us after about a year of near-daily methamphetamine. For about three months before admission she had been hearing voices and was sure her neighbors were filming her.
3. I haven't used since I got here, and I don't want to. Nurse: Her urine tests, twice a week for four months, have all been negative, and she hasn't left the grounds.
4. Nurse: Four months clean, and the voices haven't let up. She hears two women talking about her most days and keeps tape over the vent in her room.
5. Darnell: She used to be the funny one. Now she barely talks, her face doesn't move, staff have to remind her to shower, and she dropped out of the program's job training. It's been seven months since she first told me about the voices.
6. The doctor here did blood tests and a brain scan, and they were normal. I'm not sad and I've never felt high; I just don't feel much of anything.

What a skilled clinician weighs after each clue

1. Substance/medication-induced psychotic disorder, Schizophrenia, Social anxiety disorder, Paranoid personality disorder
2. Substance/medication-induced psychotic disorder, Schizophrenia
3. Substance/medication-induced psychotic disorder, Schizophrenia
4. Schizophrenia, Psychotic disorder due to another medical condition, Schizoaffective disorder
5. Schizophrenia, Schizoaffective disorder
6. Schizophrenia

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Substance/Medication-Induced Psychotic Disorder; SAMHSA TIP 42 (2020), Substance Use Disorder Treatment for People With Co-Occurring Disorders